

Paediatric-NIH Stroke Scale (Paed-NIHSS)		GET STARTED:				
		1. Access and document NIHSS score with either method: a. Dropdown bar: "Scoring Tools" → "NIH Stroke Scale" [ASAP module] b. Dropdown bar: "Flowsheets" → In search bar, enter "NIHSS". [ASAP and inpatient module] 2. Use this guide with specific instructions designed to assess young children.				
1. Perform the Ped-NIHSS in the specified order		Date:				
2. Judge the child's first attempt; do not coach		Time:				
3. Judge what is observed, not the child's intent		BP (mmHg)				
1A. LEVEL OF CONSCIOUSNESS						
For ages 4 months to 2 years, multiply the score for this item by 3, and omit scoring items 1B & 1C. - Ask 1 – 2 questions - Stimulate - Noxious stimuli	Alert; keenly responsive	0				
	Not alert but arousable by minor stimulation to obey, answer, or respond	1				
	Not alert, requires repeated stimulation, or is obtunded and requires strong/painful stimulation to make movements (not stereotyped)	2				
	Responds only with reflex motor or autonomic effects or totally unresponsive, flaccid, areflexic	3				
1B. ASK AGE AND TO POINT OUT FAMILY MEMBER. (OMIT IF AGE < 2 YEARS)						
Ask "how old are you?", give credit if child states correct age or shows correct number of fingers for age. Ask "where is X?" (X = family member present); give credit if child correctly points/gazes purposefully in the direction of the family member.	Answers both questions correctly	0				
	Answers 1 question correctly	1				
	Answers neither question correctly	2				
1C. COMMAND TO BLINK EYES & TOUCH NOSE. (OMIT IF AGE < 2 YEARS)						
	Performs both tasks correctly	0				
	Performs 1 task correctly	1				
	Performs neither task correctly	2				
2. HORIZONTAL EXTRAOCULAR MOVEMENTS.						
Only assess horizontal gaze.	Normal	0				
	Partial gaze palsy (abnormal gaze in one or both eyes but no forced deviation or total gaze paresis)	1				
	Forced deviation or total gaze paresis not overcome by oculoccephalic manoeuvre	2				
3. VISUAL FIELDS						
(> 6 years) Use finger counting (2 – 6 years) Visual threat	No visual loss	0				
	Partial hemianopia	1				
	Complete hemianopia	2				
	Bilateral hemianopia (blind including cortical blindness)	3				
4. FACIAL PALSY						
	Normal symmetrical movement	0				
	Minor paralysis (flattened nasolabial fold, asymmetry on smiling)	1				
	Partial paralysis (total or near total paralysis of lower face)	2				
	Complete paralysis of one or both sides (absence of facial movement in upper & lower face)	3				
5A. LEFT ARM MOTOR DRIFT						
Drift is scored if arm falls before 10s. For children unable to follow precise directions or uncooperative, grade power by observation of spontaneous or elicited movement	No drift, limb holds 90 (or 45) degrees for full 10 seconds	0				
	Drift, limb holds 90 (or 45) degrees but drifts down before full 10s; does not hit bed or other support.	1				
	Some effort against gravity, limb cannot get to or maintain (if cued) 90 (or 45) degrees, drifts down to bed, but has some effort against gravity	2				
	No effort against gravity, limb falls	3				
	No movement	4				
	Amputation, joint fusion	UN				
5B. RIGHT ARM MOTOR DRIFT						
Drift is scored if arm falls before 10s. For children unable to follow precise directions or uncooperative, grade power by observation of spontaneous or elicited movement	No drift, limb holds 90 (or 45) degrees for full 10 seconds	0				
	Drift, limb holds 90 (or 45) degrees but drifts down before full 10s; does not hit bed or other support.	1				
	Some effort against gravity, limb cannot get to or maintain (if cued) 90 (or 45) degrees, drifts down to bed, but has some effort against gravity	2				
	No effort against gravity, limb falls	3				
	No movement	4				
	Amputation, joint fusion	UN				
6A. LEFT LEG MOTOR DRIFT						
Drift is scored if the leg falls before 5 sec; for children too young to follow precise directions or uncooperative, grade power by observation of spontaneous or elicited movement	No drift, leg holds 30 degrees position for full 5 seconds	0				
	Drift, leg falls by the end of the 5 second period but does not hit bed	1				
	Leg falls to bed by 5 seconds, but has some effort against gravity	2				
	No effort against gravity, leg falls to bed immediately	3				
	No movement	4				
	Amputation, joint fusion	UN				
6B. RIGHT LEG MOTOR DRIFT						
Drift is scored if the leg falls before 5 sec; for children too young to follow precise	No drift, leg holds 30 degrees position for full 5 seconds	0				
	Drift, leg falls by the end of the 5 second period but does not hit bed	1				

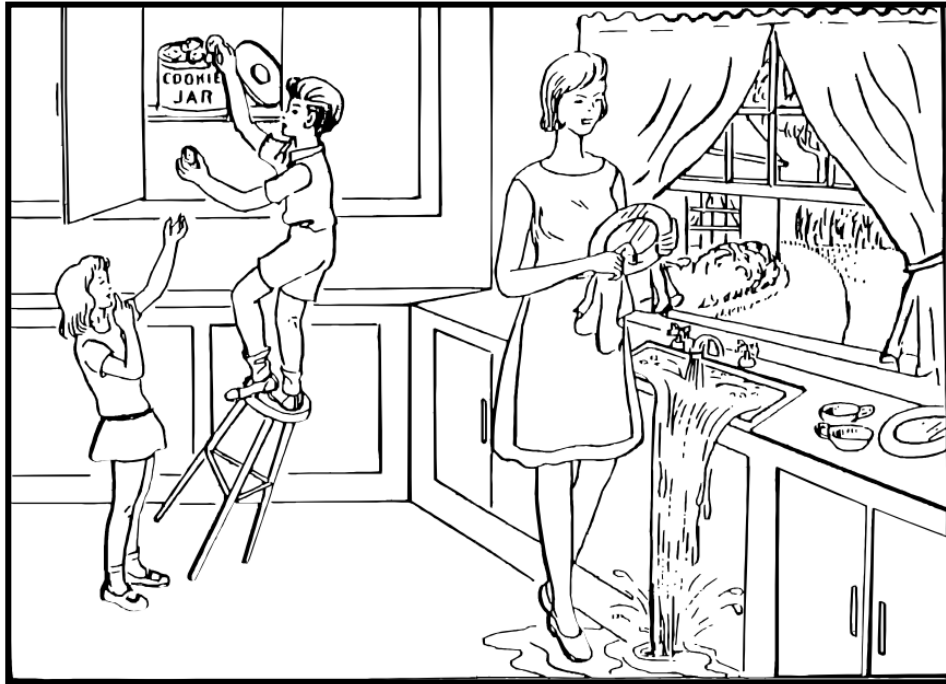
<i>directions or uncooperative, grade power by observation of spontaneous or elicited movement</i>	Leg falls to bed by 5 seconds, but has some effort against gravity	2			
	No effort against gravity, leg falls to bed immediately	3			
	No movement	4			
	Amputation, joint fusion	UN			
7. LIMB ATAXIA					
<i>If age < 5 years, or uncooperative:</i> - Reach for a toy - Kick a toy	Absent	0			
	Present in 1 limb	1			
	Present in 2 limbs	2			
8. SENSATION					
<i>Pin-prick test; for children too young or otherwise uncooperative for reporting gradations of sensory loss, observe for any behavioural response to pin prick</i>	Normal, no sensory loss	0			
	Mild to moderate sensory loss; patient feels pinprick is less sharp or dull on the affected side, or there is a loss of superficial pain with pinprick, but patient is aware he/she is being touched	1			
	Severe to total sensory loss; patient is not aware of being touched in the face, arm, and leg	2			
9. BEST LANGUAGE (Refer to page 3 for diagrams)					
<i>If age ≥ 6 years with normal language development before onset of stroke:</i> - What is happening in the picture? - Name these items - Repeat the words - Read the sentence <i>If age 2 to < 6 years, score item based on observations of language comprehension and speech during the examination</i>	No aphasia, normal	0			
	Mild to moderate aphasia	1			
	Severe aphasia	2			
	Mute, global aphasia; no usable speech or auditory comprehension	3			
4 months to 2 years: score for auditory alerting & orienting responses	Alerts to sound; spatial orientation to sound visually or by behaviour	0			
	Alerts to sound; does not have a spatial orientation to sound	1			
	Does not alert to sound	2			
	Comatose or mute patient	3			
10. DYSARTHRIA					
<i>Ask patient to read or repeat words</i>	Normal	0			
	Mild to moderate dysarthria; patient slurs at least some words and, at worst, can be understood with some difficulty	1			
	Severe dysarthria; patient's speech is so slurred as to be unintelligible in the absence of or out of proportion to any dysphasia, or is mute/anarthric	2			
	Intubated or other physical barrier (e.g., orotracheal trauma)	UN			
11. EXTINCTION AND INATTENTION					
<i>Patients with aphasia but appears to attend to both sides (normal)</i> <i>Patients with severe visual loss preventing visual double simultaneous stimulation, and the cutaneous stimuli are normal (normal)</i>	No abnormality	0			
	Visual, tactile, auditory, spatial, or personal inattention or extinction to bilateral simultaneous stimulation in one of the sensory modalities	1			
	Profound hemi-inattention or hemi-inattention to >1 modality; does not recognize own hand or orients to only one side of space	2			
4 months to 2 years	No abnormality	0			
	Either sensory or motor deficit	1			
	Both sensory & motor deficits (Patient is comatose)	2			
Total Score					

QUESTION 9: BEST LANGUAGE

1. Give sufficient time.
2. Ask if patient wears glasses.
3. Patients with visual impairment
 - a. Identify objects placed in hand.
 - b. Repeat, and produce speech.
4. Intubated patient: Ask patient to write the answer

Fluency Test

What is happening in the picture?

**Naming Test**

Name these items

**Repetition Test**

Repeat the words.

- A. Stop
- B. Stop and go
- C. If it rains, we play outside
- D. The president lives in Washington

Reading Test

Read the following

- A. Stop
- B. See the dog run
- C. Little children like to play outdoors